

SECOND AMENDED STATEMENT OF FACTS AND CONTENTIONS (FORM 9A)

WC/2024/227 - Cory Lea Shepherd (Appellant) v Workers' Compensation Regulator (Respondent). Section 550(4), Workers' Compensation and Rehabilitation Act 2003. Particulars are given by reference to the paragraph numbers of the Appellant's notice to admit facts served on the Respondent on 28 August 2026, to which the Respondent responded on 8 September 2026.

PART A

Appellant: Cory Lea Shepherd. Employer: State of Queensland (Queensland Health) - Logan Hospital Switchboard. Role: Administration Officer, Switchboard Services, classification AO3, a continuous shift working role. Injury: major depressive disorder with anxious distress (DSM-5 296.23), recorded in writing by the Appellant's treating psychiatrist on 24 October 2024 and coded DSM-5 296.23 in his report of 13 February 2025. The injury is a diagnosed medical condition arising from the Appellant's employment over the course of conduct pleaded at Part B.2 below, between approximately June 2023 and June 2024. It is not pleaded as arising from an event occurring on a single day. The date of injury is 18 June 2024, as certified by the treating general practitioner. The injury is pleaded as arising from that course of conduct, not from a single event on that day.

Particulars: paragraphs 1 to 4, 13 to 16 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

PART B - MATERIAL FACTS

Except where a paragraph is expressly identified as context only, the documentary facts identified below have been admitted to the extent recorded in the Respondent's response of 8 September 2026. Those admissions provide evidence from which the Commission is invited to determine the nature, reasonableness and causal significance of the relevant circumstances.

Pleading architecture. Each stressor is pleaded so that it can be determined on its own factual footing. Where the same admitted fact bears on more than one stressor, it is deliberately repeated. The repetition does not allege separate injuries or double-count causation; it identifies the different legal and factual functions of the same event within the composite course. The medical material is interleaved below only to show contemporaneous reporting, diagnosis, capacity and treating history. It is not used to convert a clinical history into proof of an event that is otherwise disputed.

1. Baseline - the medical record

1.1 The Switchboard receives emergency response notifications and distributes them to the response groups by category of emergency, and maintains the contact and paging information by which that is done.

Duty engaged: paragraphs 5, 7 to 8, 10 of that notice (the role description).

1.2 **Prior health.** Before 18 June 2024 the Appellant had not been diagnosed with or treated for depression. The general-practice record of 16 November 2023 (Dr Nanayakkara) records poor sleep with shift work, that the Appellant could not do shifts without a good sleep, "no psychological illness such as depression or psychosis", and mood good, with melatonin and temazepam prescribed that day. [Tab M1 - general-practice records, Our Medical Ashmore, 1 January 2023 to 1 July 2024, obtained by the Respondent under the Form 29 signed 4 July 2025; Respondent's item 11.] Relied upon as the contemporaneous record that shift work was affecting the Appellant's sleep, and of what was prescribed for it, before any claim or proceeding. Its clinical significance is a matter for the treating doctors. Private entries unrelated to the injury are redacted on the extracted pages and marked as such. No diagnosis of, or treatment for, a depressive disorder before June 2024 is recorded; no psychotropic medication was current in May 2024 (paragraph 1.3); and on the same records the Respondent's own review found that employment was a significant contributing factor to the injury (paragraph 1.10). Section 32(1) requires that employment be a significant contributing factor, not the only one.

1.3 **The referral of 16 May 2024.** The referral letter of 16 May 2024 (Dr Zhao) renews a referral to a psychiatrist, Dr Amini, for "ongoing care and management". It lists the past medical history as the history-list items "26/10/2022 ADHD" and "26/10/2022 Anxiety", and lists the medications then current, which include no antidepressant, no anxiolytic and no other psychotropic medication. [Tab M1.] No appointment with the psychiatrist was obtained on that referral.

Particulars: paragraphs 76 to 77 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.4 **Deterioration and first presentation.** The Appellant last worked a shift on 3 June 2024. The date of injury recorded on the certificates is 18 June 2024. On 28 June 2024 (Dr Slawinski) the record shows "stress at work" and "upset by people not following rules", reason for visit anxiety. On 1 July 2024 (Dr Hawes) the record shows "work stress", "been there 5 years", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies", and "causing anxiety". [Tab M1.] The events those records name are the matters admitted on 8 September 2026 and pleaded at Part B.2 below. The records are relied upon as the contemporaneous record of what was reported, before any claim decision, dismissal or proceeding; not as a finding of fact as to mechanism.

1.5 Certification. The work capacity certificates of Dr Hawes dated 1 July, 11 August and 8 September 2024, and of Dr Ki Pang dated 7 August 2024, state the date of injury as 18 June 2024, that the Appellant was first seen for this injury on 1 July 2024, and certify no functional capacity continuously from 1 July to 6 October 2024. The referral to a psychiatrist is recorded on the certificate of 8 September 2024. Review Decision 69983 records at page 17 that the certificate of 1 July 2024 indicated "there was no pre-existing factor or condition", and that "This was maintained in all later work capacity certificates". [Tab M2; Respondent's items 7 and 8.] The medication box is unticked on each certificate.

1.6 Diagnosis, 24 October 2024. At the first consultation, on the referral, the treating psychiatrist told the Appellant, and confirmed in writing at 11:45 am that day, that he was "suffering from psychological injury of Major Depressive Disorder with anxiety state"; that fluoxetine was increased to two capsules "from today" and Seroquel 25 mg commenced at night, "to restore basic needs- sleep, eating and routine". The Appellant's email of 5:12 pm that day reads "I have finally been able to see a psychiatrist today". [Tab M3; Respondent's item 9.] Read with 1.5, it records the progression from the injury certified as anxiety and stress on 1 July 2024 to a depressive disorder by 24 October 2024.

1.7 The report of 13 February 2025. Diagnosis: Major Depressive Disorder with anxious distress (DSM-5 296.23). Severity, functional effect and treatment (fluoxetine increased to three capsules daily; quetiapine 25 mg at night). The treating clinician records the origin as "workplace stress stemming from issues with management and rostering", records delayed pay as a source of financial stress, and states that "premature exposure to the workplace is more likely result in significant deterioration". [Tab M4; Respondent's item 10.] The stressors so recorded correspond to matters admitted on 8 September 2026: management and rostering, and the night-shift line; the shorter break; and pay withheld or delayed from the 5 February 2024 fortnight, uncorrected at 13 May, "claims older than 3 months" on 28 May, a claim effective 30 March recorded "Part Completed" on 30 May, and the two February claims absent from the myHR report.

Particulars: paragraphs 193, 196 to 197, 203, 210 to 212, 220, 258 to 260 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.8 The report's footer, and the author's position. The report bears a footer reading "for the only reason of clinical information and not for medico-legal use". The author's emails of 5 and 8 September 2026 state of his records "You can use them according to the need to support your legal issues", and identify the report of 13 February 2025 as "the report that captures the relevant information you have requested" on the matters in issue. [Tab M5.] The report is not relied upon for attribution among the individual events pleaded at Part B.2, nor for any matter after 24 October 2024 as a cause of the injury.

1.9 Current capacity. The Employee Capability Checklist completed by Dr Day Hong Ma on 3 July 2026 records current capacity and restrictions and the continuing effect of the injury, including "symptom exacerbation on exposure to the identified workplace stressors". [Tab M7.] Relied upon for effect and capacity only, not for causation; it post-dates 1 July 2024.

1.10 Outstanding, and the finding in the decision under appeal. The clinical records of the treating psychiatrist from 24 October 2024 were offered by the practice on 5 September 2026 and have been requested; they are not yet received and will be served on receipt. On the medical evidence then before it, the Respondent's review of 24 October 2024 found that the Appellant "sustained a personal injury of a psychological nature" and stated: "Having regard to the medical evidence, I am satisfied your employment was a significant contributing factor to the psychological injury". The claim was rejected under section 32(5), not section 32(1). [Tabs M6, M8 and M9.] The hearing is de novo and that finding does not bind the Commission; it is relied upon as an admitted document.

Particulars: paragraphs 261 to 262 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.11 The medical evidence at hearing. No medico-legal report has been, or will be, prepared for this proceeding. The Appellant relies on the treating records and documents at Tabs M1 to M4 and M7 to M9, served with the Schedule of Medical Documents Relied Upon on 9 September 2026, and on the oral evidence of the treating general practitioner, Dr Peter Hawes, and the treating psychiatrist, Dr Ravikumar Bangalore Krishnaiah, each named in the Appellant's list of witnesses filed 9 September 2026. Each will be asked to address, from his own records and the admitted facts put to him: the diagnosis; the history reported to him and when; whether the Appellant's employment, and in particular the matters pleaded at Stressors 1 to 3, was a significant contributing factor to the injury; and the non-employment matters recorded in the report of 13 February 2025. Notice of that evidence is given by this paragraph. The Appellant will apply, once hearing dates are set, for attendance notices and for leave for each practitioner to give evidence by telephone or video link at a fixed time.

2. Causative stressors (composite course, June 2023 - June 2024)

In summary. The injury: a psychological injury, diagnosed, with employment found by the Respondent's own review to be a significant contributing factor [¶¶ 261 to 262]. Stressor 1: a continuous 24/7 patient-safety function [¶¶ 2, 8, 289] carried on while database access was removed and not restored [¶¶ 40, 55]; process changes made effective

the day they were notified, with no listed record of consultation [¶¶ 51, 181, 273]; calls misdirected and the directory not corrected [¶¶ 58, 67, 89 to 93, 109 to 110]; and the manager's office hours not stated for nine months [¶¶ 71 to 73]. Stressor 2: pay wrong in four fortnights [¶¶ 185 to 189]; Payroll's direction of 3 May 2024 not actioned at 13 May [¶ 193], still awaited on 21 May [¶ 195], lodged 28 May and recorded "Part Completed" [¶ 203]; and a paid leave entitlement declined twice on attachments the Respondent admits were present [¶¶ 116, 118, 134]. Stressor 3: a seven-hour break rostered in that role [¶ 284] against a minimum of ten hours, or eight by written agreement [¶ 285], the agreement applying only to staff-initiated swaps and no swap alleged [¶¶ 225, 234]; the fatigue enquiry answered after 23 days with a refusal [¶¶ 247, 249]; no fatigue risk management at the Switchboard before 30 June 2024 [¶¶ 264, 269 to 271]; and the Respondent's own review finding that the rostering was unreasonable management action [¶ 260].

STRESSOR 1 - THE CONDITIONS IN WHICH THE WORK WAS CARRIED ON

(a) The duty to keep the contact information accurate. A key responsibility of the position is to collate information and maintain the Omnivista database and SharePoint so that the information held within Switchboard Services is accurate and appropriate. [¶ 6]

A further key responsibility is to support hospital staff in the allocation, coordination and fault repair of all pager units, including maintaining the database and registers with accurate and current information. [¶ 7] That duty is the duty by reference to which each of (b) to (h) below is pleaded.

(b) The removal of the means of performing that duty, 18 July 2023. On 18 July 2023 at 12:56 pm the Switchboard Telecommunications Coordinator wrote to the Appellant and the Switchboard team explaining that her role was a temporary project role focused on fixing the database. [¶ 39]

She stated: "While I am fixing up the database and all of its entries, I will be removing everyone's access to the database." [¶ 40]

She stated that she had removed the Contact & Number Changes book out of the room, preferring all changes to be sent to her email. [¶ 45]

The Respondent's own review decision records the same matters. [¶¶ 46 to 48]

The Respondent does not allege that access to the database was restored to the Appellant at any time before 18 June 2024. [¶ 55] The duty at (a) remained on the position throughout that period.

(c) The repair path, and its availability. The Coordinator's stated working hours were "every Tuesday and second Monday, 8:00 16:00". [¶ 41] A person wanting an entry updated was to "contact me directly, either via email or extension". [¶ 42] If an entry was needed on a day she was not present, and was urgent, it was to be sent to the Switchboard Manager. [¶ 43]

A request made "after hours (overnights, on the weekend or public holiday)" would "have to wait until either Chloe or myself are back". [¶ 44]

The Appellant pleads no characterisation of any person. The availability of those two persons is pleaded because it determined how long an incorrect entry remained uncorrected.

(d) The criticality of the function, as admitted by the Respondent. On 18 February 2026 the Respondent responded to an earlier notice to admit facts in this proceeding. [¶ 283] It admitted that "Maintaining accurate contact details for medical staff is a critical function of the Switchboard to ensure effective clinical handover and patient safety". [¶ 289] In admitting that paragraph it added "and says that there was a procedure in place for this to occur", and it denied a further paragraph on the ground that there was already a procedure in place. [¶¶ 290 to 291] The procedure in place from 18 July 2023 is the procedure pleaded at (c) above. Its operation between 2 and 20 May 2024 is pleaded at (g) and (h) below.

(e) The further changes to process, April and May 2024. On 15 April 2024 at 12:39 pm the Switchboard Manager wrote to Logan Switch, copied to the Director and the Switchboard staff, with the subject "Afterhours Oncall Process - Switchboard". [¶ 49] That email stated that the process during office hours remained the same and that staff were to contact her through switch, the office or her mobile, and: "You will see that Ellen and myself have added Afterhours on call, the days that are highlighted in purple show who is on call after hours." [¶¶ 50, 52] It stated: "This new process is effective from today." [¶ 51]

She signed it as "A/Switchboard Manager". [¶ 53]

The review decision records that on 19 April 2024 she emailed the team stating that due to errors being made with respect to data entry a new process was to be followed which included more checks to ensure accuracy of data entry. [¶ 54]

On 13 May 2024 at 4:29 pm the on-call roster for the following fortnight was circulated to the Switchboard team. [¶¶ 111 to 112]

From 14 May 2024 the process for notifying unavailability for a rostered shift was also directed through the same person, both emails identifying the Switchboard as a means of contacting her. [¶¶ 162 to 165]

The Respondent does not allege that the Appellant was informed before 14 May 2024 that any other means of notification was required. [¶ 166]

(f) The obligation to consult, and the absence of any record of consultation. Clauses 3.2.3 to 3.2.5 of the Queensland Public Health Sector Certified Agreement (No. 11) 2022 provide that the requirement of consultation is never to be treated perfunctorily or as a mere formality; that consultation involves more than a mere exchange of information and requires participants to contribute to the decision-making process "not only in appearance, but in fact"; and that the process requires the exchange of timely information so that the parties have "an actual and genuine opportunity to influence the outcome, before a final decision is made". Clause 4.1.4 requires, for significant organisational change including major alterations to current service delivery arrangements, a business case tabled for consultation; it is relied upon only to the extent the change was of that kind. The change notified on 15 April 2024 was stated to be effective the day it was notified. [¶ 51]

The Respondent's amended List of Documents records no document recording consultation with Switchboard operators before that change. [¶ 273]

The Respondent does not allege that agreement under clause 6.2 of the Award was obtained, or that a ballot of affected employees was conducted, before it. [¶¶ 180 to 181]

The employer conducted that process on other occasions. Its Consultation outcome document defines agreement as the consent of a majority of affected employees, and records that seventeen employees were invited to vote, seventeen votes were received, and the proposal was accepted. [¶¶ 167 to 170, 176 to 177]

On 20 May 2024 at 4:30 pm the Switchboard Manager wrote: "Taking note of your recommendation, we can also put the updated procedures out to the team for consultation before implementing." [¶ 101]

(g) What arrived at the console: the misdirected emergency calls, 2 to 9 May 2024. A key responsibility of the position is to participate in the Emergency Response process by receiving emergency response notifications and distributing them to the appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes; to maintain call queues to a minimum at all times; and to operate under pressure, particularly where high volume call traffic is concerned. [¶¶ 5, 8, 10]

The Appellant will give evidence that the console took in the order of 270 to 440 calls on an eight-hour shift, and that emergency codes were received and paged through it throughout the day (Stressor 3(h) below). That evidence is his own and is not admitted.

On 3 May 2024 at 3:06 pm the MASPER Registrar wrote to the Switchboard Manager with the subject "Switchboard issues 2nd and 3rd May 2024", listing five occasions on 2 and 3 May. [¶¶ 56 to 57] Of the occasion at 14:46 on 3 May the email records: "MET call team called x5290 asking where MET call was located 'VHUB' switchboard could not tell them where VHUB was. Had to be redirected by MASPER". [¶ 58]

On 8 May at 5:28 pm she wrote again, listing four further occasions, recording of those on 8 May that a call was "incorrectly put through to MASPER" on each occasion. [¶¶ 59 to 61] Those nine occasions fell while the arrangement notified on 15 April 2024 was in force. [¶ 68]

The first reply was sent on 9 May at 9:20 am, and asked the reporting clinician to confirm her business hours because they were "not provided on the rosters", noting that her dect phone was "currently switched off". [¶ 65]

That reply was sent five days, eighteen hours and fourteen minutes after the first email. [¶¶ 62 to 64]

At 10:15 am the same day the Switchboard Manager wrote to Logan Switch introducing a new process and referring to "many ongoing issues raised by the MASPER and the medical department about calls being transferred to the wrong medical teams". [¶¶ 66 to 67]

The Respondent does not allege that any communication was sent to Logan Switch or to the Switchboard staff concerning those occasions at any time after 3 May 2024 and before 10:15 am on 9 May 2024. [¶ 69]

(h) What arrived at the console: the Integrated Respiratory Service, 15 to 20 May 2024. On 15 May 2024 at 11:47 am an Administration Officer of the Integrated Respiratory Service wrote to Logan Switch, marked of High importance: "Could you please amend your number registry/directory to show #8768 belongs to the Integrated Respiratory Service... We are not Respiratory Medical OPD and we do not have any doctors working out of this area." [¶ 89]

The Respondent does not allege that any response was made to that email before 20 May 2024. [¶ 90]

On 20 May at 11:03 am she wrote again, marked of High importance: "Just a courtesy reminder, we continue to get calls put through to us for Respiratory Medical Outpatients... we can not help patients or other clinical staff with OPD issues." [¶¶ 91 to 92]

The Respondent does not allege that any response was made to that email before 2:05 pm that day. [¶ 93]

At 2:05 pm, from the Logan Switch account and within the office hours stated to all Switchboard staff on 17 May 2024, the Appellant wrote identifying the cause as modifications made to the document "Outpatients Department - Clinic contact Details" on 22 February 2024, and recommending a modification and review of that document. [¶¶ 94 to 96] That email was sent three hours and two minutes after the Service's second email, and five days, two hours and eighteen minutes after its first. [¶¶ 97 to 98]

The Respondent does not allege that the Appellant was rostered to work after 2:05 pm that day. [¶ 99] The reply at 4:30 pm stated: "Thank you for bringing this to my attention however this task was being actioned. I had discussed with Richard this morning about the update of outpatients respiratory/medical." [¶¶ 100, 104] It was sent two hours and twenty-five minutes after the Appellant's email, and two hours after the conclusion of the office hours stated to all Switchboard staff on 17 May 2024. [¶¶ 102 to 103]

The Respondent's amended List of Documents records no document created on 20 May 2024 recording that discussion, and no communication sent by either named person to the Service on that day; and the Respondent does not allege that the other named person communicated with the Service on that day. [¶¶ 105 to 108]

The Respondent does not allege that the contact-details document was amended on or before 20 May 2024, or that the Switchboard staff were notified of the modifications made to it on 22 February 2024. [¶¶ 109 to 110]

(i) The office hours of the manager to whom the position reports. The position reports to the Switchboard Manager, and the role description names that person as the contact for the position. [¶¶ 3 to 4]

On 23 August 2023 at 2:18 pm she wrote to Logan Switch with the subject "What's Chloe's Hours?!": "I am fulltime and work flex hours - You may have noticed recently I am starting at different times in the morning and finish at different times in the afternoon", and "My hours would range from starting between 6-9am and finishing 2-5pm." [¶¶ 70 to 72]

The Respondent does not allege that fixed office hours were stated to the Switchboard staff at any time between 23 August 2023 and 17 May 2024. [¶ 73]

On 15 May 2024 at 1:15 pm the Appellant wrote to the Manager and Logan Switch, copied to the Switchboard staff, the Director and LBH_HR, asking that she share her office hours so the department could be aware of her regular schedule, noting inconsistency in arrival and departure times, and asking that "any directives and changes within the department is made in consultation with the team". [¶¶ 74 to 75]

At 6:23 pm the Director replied, copied to the Manager and marked of High importance, stating that his email "did not demonstrate our iCARE2 value of Respect and did not comply with our Code of conduct", and asked him to retract it. [¶¶ 76 to 77]

At 7:09 pm the Appellant replied that "Requesting clarity on business hours is a reasonable" request. [¶ 78]

On 17 May 2024 at 9:30 am the Manager wrote to the whole department, headed "Switchboard Manager - On call and Hours", stating that her hours "can vary", that "I will be in later between 0800-830am. Otherwise my hours are from 06:30-14:30", that "I will not be in the office until around 11am on Monday 20.05.2024", and that "Moving forward so communication is clear for the team, I will be sending an email to switch to advise of any change to my office hours for the week". [¶¶ 81 to 84, 113] Those stated office hours concluded at 2:30 pm. [¶ 88]

On 21 May 2024 at 2:53 pm the Director replied that she would follow up on the issues raised, and asked "can I ask was there one or more particular changes and/or directives where you had concerns about consultation and communication?" [¶¶ 79 to 80]

On 18 June 2024 at 8:58 am the Manager wrote to Logan Switch: "I am taking today off, and hopefully will be back tomorrow", and "I am sorry I haven't been there for you all over the past week." [¶¶ 85 to 87]

(j) The Communication Book, 6 June 2023. A key responsibility of the position is to maintain discretion and exercise judgement where necessary to resolve problems within the scope of the role, in situations where precedence has not been set and procedures are not defined. [¶ 9]

At 9:57 am on 6 June 2023 the Switchboard Manager wrote to Logan Switch that the book "is not used to simply put your point across or make any indirect comments towards the team". [¶¶ 148 to 149]

At 4:05 pm she wrote to the Director: "I was not aware of who put this entry in at the time but I took it out last week", and that she felt it was being used as a "burn book" more than a professional tool. [¶¶ 145 to 147]

The Respondent's pleading admits that pages were removed on or around 6 June 2023, and says the event was reported to the Director by email that day. [¶¶ 143 to 144]

On 18 February 2026 it admitted that a page was removed from the workplace communications book, and did not admit that the page contained the Appellant's handwriting "because the respondent does not have a copy of the page". [¶ 288]

The Respondent does not identify the entry removed or its author, does not allege that the removed page has been located or that any copy exists, does not allege that any entry made by the Appellant remains in the book, and does not allege that the book ceased to be in its possession during his employment. [¶¶ 150 to 153]

The Respondent has admitted that it does not have a copy of the removed page. [¶ 288]

(k) The matters raised in August and September 2023, and the response. On 7 August 2023 at 12:21 pm the Appellant wrote: "Please give me some space and stop with any further communication as I have had enough of it." [¶ 155]

At 1:43 pm the Manager replied, apologising for the rostered shift and confirming it was a rostering error, and asking "Would you like me to roster you off tomorrow Tuesday 8th 0700-1500 to give you the required rest period". [¶¶ 156 to 157]

At 3:13 pm the Director replied. [¶ 158]

On 8 August 2023 at 4:15 pm the Director wrote regarding an additional shift to replace the shift not worked due to a rostering error. [¶ 28]

On 29 August 2023 she sent HR Policy E12, and on 4 September 2023 the Appellant replied. [¶¶ 160 to 161]

On 31 August 2023 he provided a written application headed "Request to Increase Working Hours to Full Time Rotational Roster", expressing willingness to take on additional night shifts. [¶¶ 30 to 31]

On 4 September 2023 he confirmed that he was able and willing to work any roster presented to him including the full 24-hour rotational schedule, that he had no issues with shift work, and attached a draft roster spreadsheet. [¶¶ 33 to 35]

On 27 September 2023 the Manager wrote with the subject "Approved - Permanent Full Time FTE", stating she was happy for him to commence full-time hours from 16 October 2023. [¶¶ 36 to 38]

On 8 August 2023 he had confirmed that he would work the replacement shift and was looking to pick up another. [¶ 29] On 8 September 2023 the Director wrote that she was "glad to hear things seem to be going well with Chloe" and glad that he had applied for the additional shifts through the recent EOI. [¶ 36]

(l) The special pandemic leave application, February 2024 - the sequence. The review decision records that on 20 February 2024 the Appellant messaged the Switchboard Manager stating he had tested positive for COVID-19, and that she "requested that you send it to her so she could submit it in MyHR". [¶ 136]

The Respondent pleads that she asked him to submit a Special Pandemic Leave request via myHR himself and attach a statutory declaration or photo evidence of his test. [¶ 132] The myHR submissions report records the initiator of that Smart Leave Request as the Appellant. [¶¶ 140 to 141]

The Respondent does not allege that any other Switchboard employee was required to submit such a request personally through myHR in February 2024. [¶ 142] The leave request history for Process Reference 15480560 records the Appellant creating a draft at 11:24:48 on 20 February 2024 and submitting the request at 11:41:27 that morning. [¶¶ 114 to 115]

It records the request being declined at 12:23:43 on 21 February 2024; submitted again at 06:07:32 on 28 February; declined again at 09:06:07 on 29 February; submitted a third time at 11:07:28 that morning; and approved at 11:21:03. [¶¶ 116 to 120]

It records the Appellant as having submitted the request on three occasions and the Manager as having declined it on two, and records the Director as approving it at 16:05:08 on 1 March 2024. [¶¶ 121 to 123] The approval followed the third submission by thirteen minutes and thirty-five seconds; the interval from draft to final approval was ten days, four hours, forty minutes and twenty seconds. [¶¶ 124 to 125] Two reasons for the declines appear on the record and they are not the same. The review decision records that on 21 February the Manager messaged that she had to decline because the minimum period for COVID leave was five days for a positive staff member, that she noted the statutory declaration provided was sufficient evidence, and that she asked him to resubmit his leave days. [¶¶ 137 to 138]

It records the Appellant messaging that he was confused about why his amended request was declined, and that he had done everything as required. [¶ 139]

The Respondent pleads that she declined the application because on her assessment the required statutory declaration was not attached; that "a review indicates that in fact, the attachments were present on the appellant's submission"; and that this was human error on the background of high work demands. [¶¶ 133 to 135]

(m) The authority to decide that application. The power to approve paid special pandemic leave was sub-delegated by an Instrument of Human Resource Sub-Delegation signed by the Health Service Chief Executive on 23 November 2022 and effective from 5 December 2022. [¶ 127]

That Instrument states: "This Instrument does not permit the further sub-delegation of the powers outlined in (a) above." [¶ 128]

In the leave request history the Switchboard Manager is identified as "Reviewer" and the Director as "Manager". [¶ 126]

The Respondent does not allege that either was a Band 9 delegate at any relevant time in February or March 2024, and does not identify the person who exercised the power sub-delegated by that Instrument in respect of Process Reference 15480560. [¶¶ 129 to 131]

(n) Public interest disclosure - chronology only. On 13 May 2024 the Appellant lodged a corrupt conduct complaint regarding clinical risks. The Ethical Standards Unit formally determined this constituted a Public Interest Disclosure (Admitted Fact: Form 24, para 20, admitted 18 February 2026). The content is not set out.

(o) Workplace representation. The Respondent has admitted that the Union Encouragement Policy QH-POL-248 requires managers to take a "positive, supportive role" in relation to workplace delegates, and admitted that the Appellant sent a text to the Manager at 5.03 pm that he was "just putting his hand up", saying that this was not a formal notification. [¶¶ 293 to 294]

On 3 November 2025 an Organiser of Together Queensland wrote to the Appellant and two colleagues congratulating them, and the Appellant was endorsed as a workplace delegate on or about that date. [¶¶ 178 to 179]

(p) What the employer states about its own systems and records. A key responsibility of the position is to follow defined service quality standards and occupational health and safety policies and procedures relating to the work being undertaken, to ensure high quality, safe services and workplaces. [¶ 12]

By letter of 5 June 2026, reference K-LM26/729, signed by the Chief Executive and addressed to the Commission, Metro South Health stated that the requested fatigue risk management documents "do not exist", that mandatory Fatigue Risk Management System training "only applies to health practitioners and clinical assistants" and that Switchboard staff are non-clinical. [¶¶ 263, 265] It stated that "The implementation of fatigue risk management assessment at Switchboard Logan Hospital occurred after 30 June 2024". [¶ 264] It stated that "there have been no 'consequential' changes to operating procedures over the period requested". [¶ 266] It stated that "All employee complaints relating to Logan Hospital Switchboard operational errors are made directly to the Line Manager of Switch Board and managed solely via email or verbally with the complainant." [¶ 267] It stated that "a spreadsheet of recorded MET calls is available for the period 17-18 March 2024". [¶ 268]

The Respondent does not allege that any fatigue risk assessment was conducted, that any fatigue risk management training was provided, that fatigue risk management assessment was implemented at the Switchboard, or that any change was made to the operating procedures of the Switchboard as a consequence of any employee complaint, at any time before 30 June 2024. [¶¶ 269 to 272]

(q) Matters pleaded as context only. The Respondent does not allege that the Appellant was subject to any disciplinary process, or that his work performance was the subject of any formal performance management process, before 18 June 2024, nor does it describe any communication with him before that date as a warning. [¶¶ 300 to 302] The matters at (j), (k), (l), (m), (n) and (o) above, and at paragraph 3 below, are pleaded as part of the sequence of matters the Appellant raised and the responses he received. They are not relied upon as independent causative circumstances within Stressor 1. Where an admitted fact from that sequence is separately repeated under Stressor 2 or Stressor 3, it is relied upon there only for the distinct function stated under that stressor. The causative circumstances relied upon in Stressor 1 are those at (a) to (i) and (p).

Medical linkage - Stressor 1. Tab M1 records the 28 June 2024 presentation as "stress at work" and "upset by people not following rules". Tab M4 records the treating psychiatrist's later account of origin as workplace stress involving management and rostering. Those records are relied upon as contemporaneous and treating evidence of reported history, diagnosis and effect. They are not relied upon to prove the occurrence of the events in Stressor 1; those events are proved, to the extent relied upon here, by the admitted facts and identified employer documents.

STRESSOR 2 - REMUNERATION

(a) The basis of the entitlement. The position is a continuous shift working role over the full 24-hour period, seven days a week, and under mandatory requirements the occupant must be able to work a roster which covers multiple shifts over a 24/7 period. [¶¶ 2, 13]

(b) Public holidays on which the Appellant was not required to work. The review decision records that section 23 of the Award provides that where an employee in receipt of the additional week's leave prescribed by clause 19.2(a) is rostered off on Easter Saturday, Easter Sunday, Show Day or Labour Day, that employee is to be paid an additional day's wage or granted a day's holiday in lieu. [¶ 241] As at 1 May 2024 payroll was "still reviewing your entitlements regarding public holidays not required arising while you were a part-time employee". [¶ 255] The Manager confirmed "that since your commencement of full-time employment, all payments for public holidays not required had been processed". [¶ 256]

(c) The correction sought, and where each direction sent it. The Respondent's disclosure contains an email thread with the subject "Corey Shepherd 388372 Pay issues", the first message being from PayrollMetroSouth on 3 May 2024 to the Manager, copying the Appellant, bearing the signature block of a Client Service Officer of the Metro South Payroll Team. [¶¶ 182 to 184] That message identifies four fortnights requiring correction and states: "Please submit

an AVAC to correct these shifts for each fortnight so Cory is paid corrected and his RDO balance will then be amended." [¶¶ 185 to 190]

On 10 May 2024 at 2:21 pm the Appellant wrote to PayrollMetroSouth and the Manager. [¶ 191]

On 13 May 2024 at 8:21 am Payroll replied to the Appellant: "I cannot see that any of the issues below have been corrected. Please speak to your Line Manager to have them corrected with an AVAC submitted through My HR". [¶¶ 192 to 193]

On 21 May 2024 at 12:33 pm the Manager wrote that she was "still I am waiting payroll confirmation". [¶¶ 194 to 195]

On 28 May 2024 at 8:36 am she wrote: "Please find attached Validation of claims older than 3 months, please sign and return to be as soon as possible so I can escalate for delegate approval." [¶¶ 196 to 197]

(d) Who could submit the correction. The myHR submissions report records seven submissions for the Appellant in the period 1 February to 31 May 2024, of which five were Attendance Variation and Allowance Claims. [¶¶ 198 to 199]

It records the initiator of each of those five as the Manager, and the initiator of no such claim in that period as the Appellant. [¶¶ 200 to 201]

(e) The state of the record, and the Respondent's pleaded position. The report records a claim with process number 16328886 submitted on 15 May 2024 and completed the next day. [¶ 202]

It records a claim with process number 16450619 submitted on 28 May 2024, with an effective date of 30 March 2024 and a processing date of 30 May 2024, with a status of "Part Completed", and records the status of every other such claim in the period as "Completed". [¶¶ 203 to 204]

It records the process numbers and submission dates in the period, and in it process numbers increase as submission dates increase. [¶¶ 205 to 206]

The Respondent pleads that "any discrepancies or errors were remedied in a timely manner" and that "there are no outstanding underpayments for the appellant", and that the Manager submitted the AVAC on 28 May 2024. [¶¶ 207 to 209]

Neither AVAC PRN 15397775 nor AVAC PRN 15605601 appears in the myHR submissions report. [¶ 210]

On 18 February 2026 the Respondent admitted the Payroll instruction of 3 May 2024, the fact that the Manager submitted the AVAC on 28 May 2024, and the email of 21 May 2024 stating that she was waiting for payroll confirmation. [¶¶ 296 to 298]

(f) Paid Special Pandemic Leave - the same admitted sequence relied upon as an employment entitlement. The power to approve or not approve paid Special Pandemic Leave for the COVID-19 event was sub-delegated by the Health Service Chief Executive to a Band 9 delegate, and the Instrument states that the power could not be further sub-delegated. [¶¶ 127 to 128] On 20 February 2024 the review decision records the Manager asking the Appellant to send his material to her so she could submit it in myHR; the leave history records the Appellant creating the draft and submitting the request that morning. [¶¶ 114 to 115, 136] The history records the request being declined on 21 February, resubmitted on 28 February, declined again on 29 February, submitted a third time at 11:07:28 that day and approved at 11:21:03. [¶¶ 116 to 120] It records the Appellant as making three submissions, the Manager as declining twice, the Manager as "Reviewer" and the Director as "Manager", with the Director recording final approval on 1 March 2024. [¶¶ 121 to 123, 126] The review decision records that on the first decline the stated reason was the minimum period for COVID leave while the statutory declaration was said to be sufficient; it records the Appellant saying he was confused by the second decline and had done everything required. [¶¶ 137 to 139] The Respondent's pleading states that the later decline was because the statutory declaration was assessed as not attached, but that review showed the attachments were in fact present, and pleads human error against a background of high work demands. [¶¶ 133 to 135] The Respondent does not allege that either recorded approver was a Band 9 delegate at the relevant time and does not identify the person who exercised the sub-delegated power for Process Reference 15480560; nor does it allege that any other Switchboard employee was required personally to submit such a request through myHR in February 2024. [¶¶ 129 to 131, 142] This sequence is deliberately repeated from Stressor 1(l)-(m): in Stressor 2 it is relied upon as the handling of a paid employment entitlement that required repeated submissions and a decision process the Appellant could not complete by himself, not as a separate injury.

(g) Rostering practices raised with Human Resources. On 20 May 2024 at 4:07 pm the Director wrote to LBH_HR: "I am just following up on this query I raised a little while ago in relation to a email I had received from a staff member about our rostering practices. Thus if someone might be able to give me a call regarding these staff concerns, that would be great." [¶ 222] That email attached the document "qh-gdl-401-3.3", the Queensland Health Fatigue Risk Management Systems Implementation Guideline. [¶ 223]

Medical linkage - Stressor 2. Tab M1 records the 1 July 2024 report of "work stress" and that pay was withheld at times and overtime was not processed. Tab M4 records the treating psychiatrist's later history of delayed pay as a source of financial stress. The medical records are relied upon for what was reported and for clinical course; the remuneration events themselves are proved by the admitted payroll, myHR and pleading facts set out above.

STRESSOR 3 - THE ROSTER AND FATIGUE

(a) The rostering history, and the Appellant's proposals. On 7 August 2023 the Manager acknowledged a rostering error and offered to roster the Appellant off the following day "to give you the required rest period". [¶¶ 156 to 157]

On 18 February 2026 the Respondent admitted that the Director stated in an email dated 7 August 2023 that there had been a rostering error that was accidentally made. [¶ 287] The Appellant applied in writing to increase his hours to a full-time rotational roster, confirmed he was able and willing to work any roster presented to him, expressed willingness to take additional night shifts, and provided a draft roster spreadsheet. [¶¶ 30 to 31, 33, 35]

On 8 August 2023 the Director wrote regarding an additional shift to replace the shift not worked due to a rostering error. [¶ 28]

(b) The rostering errors acknowledged, and the decision withheld. On 26 April 2024 at 1:52 pm the Director wrote with the subject "Roster Concerns", referring to a meeting on 16 April 2024 at which the Appellant had raised a roster line ending with night shifts followed by three days off and then returning to night shifts, and stating that the Manager "was working to fix this error and would get in touch with you about what alternative shifts she could offer". [¶¶ 211 to 212]

The Respondent does not allege that she got in touch with him about alternative shifts following that meeting. [¶ 213]

On 1 May 2024 at 1:18 pm the Appellant referred to section 4, clause 10.4.1 of the Operations Manual, which "includes a toolkit required by management to manage fatigue and implement appropriate protocols and adhere to the workplace health and safety act", and stated: "I forwarded the toolkit to Chloe last week for review and action but have yet to receive feedback". [¶¶ 214 to 215]

The Respondent does not allege that the toolkit was reviewed or that any feedback was provided before 18 June 2024. [¶ 216]

On 8 May 2024 at 9:08 am the Director replied that she was following up with HR for further advice and would get back to him as soon as possible. [¶ 217]

On 10 May 2024 at 2:08 pm she wrote to Human Resources forwarding his email, stating: "I think he is trying to raise that he has concerns over how his manager is rostering for the Switchboard team and how it is impacting on staff fatigue, or more specifically his fatigue." [¶ 218]

She stated that she and the Manager had run through the Roster Risk Assessment Matrix and "we say at best there would be a rating of 11 which is moderate". [¶ 219]

She stated: "I acknowledge there has been a few rostering errors made by Chloe with regards to Cory's line in past rosters". [¶ 220]

She stated: "Cory's roster will not be considered, however Cory is not yet aware of this as we cannot advise what is the roster we are planning on going with before we send out the implementation plan for this business case for change". [¶ 221] Eleven days later, on 21 May 2024, she asked him which particular changes or directives had concerned him. [¶ 80]

(c) The Respondent's pleaded position on the roster. On 18 February 2026 the Respondent denied paragraphs of the Appellant's earlier notice on the stated grounds "because the comparator used does not represent a true comparator in the circumstances and because the roster was equitable". [¶ 299]

The Consultation Paper of November 2024 states that the proposed roster "is intended to introduce a more equitable roster for Switchboard Services, Logan Hospital staff". [¶ 171]

It states that the proposed roster includes "Redistribution of nights for greater equity based on FTE" and a "Rotational roster for predictability and equity to all staff ensuring fair distribution of penalties". [¶¶ 172 to 173]

It states that it includes an additional eight-hour Saturday shift to assist with call volume and the extension of a five-hour weekday shift to eight hours. [¶¶ 174 to 175]

(d) The shifts of 17 and 18 March 2024. The Appellant was rostered to finish at 23:00 on 17 March 2024 and to commence at 06:00 on 18 March 2024, a break of seven hours. [¶ 284] The shift commencing on 18 March fell on a Monday, the first shift of the working week. [¶ 227]

The Respondent does not allege that those consecutive shifts arose from a staff-initiated shift swap. [¶ 234]

The review decision records the Appellant's submission that the 8-hour agreement "was no longer fit for purpose" since his transition, and the surrounding matters as to that agreement. [¶¶ 17 to 25]

(e) The rest recorded between those shifts. The seven-hour figure is the interval between the rostered finish at 23:00 on 17 March 2024 and the rostered commencement at 06:00 on 18 March 2024. The review decision records the Appellant's response of 9 August 2024 that the break "did not include travel time" and that, with travel factored in, the break between shifts would have been less than five hours. [¶ 251] This pleading relies on that admitted record. The separate estimates of wakefulness, travel, sleep and workload are not included in the admitted-fact core.

(f) The standard, as admitted. The review decision records that the employer's own response included an extract of the Award stating that employees must be provided with a break of not less than ten hours between the termination of one shift and the commencement of another, and that eight hours applied instead of ten only in specific circumstances. [¶ 257]

The Award itself, clause 15.1(d), provides: "Employees must be allowed a break of not less than 10 hours between the termination of one shift and the commencement of another shift, except that 8 hours applies instead of the 10 hours in any of the following circumstances: (i) for the purposes of changing shift rosters by shift workers who rotate from one shift to another; or (ii) in any other case agreed upon between the employer and the employee concerned." Clause 18.10, on which the Respondent relies at paragraph 24(b) of its statement [¶ 237], is headed "Fatigue leave/rest period after overtime" and applies where overtime is worked. [Hospital and Health Service General Employees (Queensland Health) Award - State 2015]

On 18 February 2026 the Respondent admitted that "The Employer's 'Fatigue Risk Management Policy' and the relevant Award require a minimum break of 10 hours between shifts, or 8 hours by written agreement". [¶ 285] It denied a further paragraph of that notice on the ground that in June 2020 the Appellant signed an agreement allowing an 8-hour break between shifts. [¶ 286]

(g) The agreement of 17 June 2020, and its scope. By email of 7 July 2026 a Senior Consultant, Human Resources wrote that the rostering "currently provides more than" the required break, and acknowledged the 8-hour agreement signed on 17 June 2020 "which allows you to work with only an 8 hour break, however this is only applied where staff initiated shift swaps have occurred". [¶¶ 224 to 225]

The Respondent pleads that the shift was separated by only a 7-hour break, that this was a result of human error and not intentional or repeated, that in June 2020 the Appellant signed an agreement allowing an 8-hour break, and that "the appellant could refuse shifts at anytime". [¶¶ 226, 232 to 233] The Respondent's outlines of evidence served 24 September 2026 (Part B.6) give notice that its witnesses will say the break "was the result of human error", and that the advice returned within payroll was that "what had occurred was better characterised as a rostering practice issue for the line manager rather than an unpaid entitlement".

The role description states, under mandatory requirements, that the position is a continuous shift working role and that the occupant must be able to work a roster which covers multiple shifts over a 24/7 period. [¶ 13]

(h) The emergency workload on those shifts. Metro South Health has stated in writing that "a spreadsheet of recorded MET calls is available for the period 17-18 March 2024". [¶ 268] A key responsibility of the position is to participate in the Emergency Response process by receiving emergency response notifications and distributing them to the appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes; and to support the allocation, coordination and fault repair of all pager units for the hospital. [¶¶ 7 to 8] The Switchboard staff are, on the employer's own statement, non-clinical. [¶ 265]

(h)(i) The register: not admitted, to be proved. The matters in this sub-paragraph are taken from the document at Tab 31 to the notice to admit, the "2024 Emergency Code Register" (March 2024 sheet), and from the Appellant's own records. The Respondent did not admit paragraphs 228 to 231 of that notice. They are not relied upon as admissions. The Appellant will prove them at hearing by tendering the register, by the employer's spreadsheet referred to at [¶ 268], and by his own evidence as the operator.

On the Appellant's reading of that register, it records for 17 March 2024, the shift that ended at 23:00 [¶ 284], the following entries: 3:57 Code Grey, ED lounge; 5:19 MET call, Ward 5A bed 27; 7:48 Code Grey, ED corridor near short stay; 13:48 MET call, Ward 3B bed 2; 14:13 Code Grey, ED adult acute bed 15; 19:16 MET call, Ward 2H bed 7; 19:20 Code Blue, Ward 2H bed 7; 21:16 MET call, Ward 2Q bed 5. Eight entries in the day; three of them, including the Code Blue, in the evening of the shift ending at 23:00.

For 18 March 2024, the shift that began at 06:00 [¶ 284]: 8:48 Code Grey, QAS triage airlock; 9:20 MET call, Ward 6A bed 8; 11:31 MET call, Ward 3DR bed 14; 11:32 cancelled MET call, Ward 3DR bed 14; 11:35 Neonatal MET call, Theatre 2; 13:09 Neonatal MET call, Birth Suites 5; 14:38 MET call, Ward 3DR bed 11; 14:45 and 14:55 Code Grey, QAS triage; 19:15 and 19:34 Code Grey, ED QAS ramp. Eleven entries in the day; six between 06:00 and 14:00, including two neonatal MET calls.

For 19 March 2024, the day on which 7.60 hours of sick leave was taken [¶ 235]: 0:17 MET call, Ward MIU bed 21; 1:41 Code Grey, ED ambulatory care; 9:27 Paediatric MET call, Ward CIU bed 16; 10:28 simulation MET call, Ward 7C bed 3; 11:30 MET call, Ward 2I bed 20; 11:34 MET call, Ward 2I bed 20, annotated "CALLED VIA SWITCHBOARD" [sic]; 12:36 MET call, Ward 2I bed 12; 13:42 MET call, Ward 3DR bed 11; 16:28 MET call, Ward 3DR bed 11, cancelled at 16:29; 16:48 Code Grey, ED mental health lounge; 17:23 MET call, Ward 3DR bed 11; 17:31 Code Grey, ED ambulatory care room 2; 18:45 MET call, Ward 5B bed 7; 21:28 Code Grey, ED mental health lounge; 22:25 Code Grey, ED QAS triage. Sixteen entries in the day; between 06:00 and 14:00, six, of which five were MET calls, one of them recorded as called through the Switchboard.

(h)(ii) What the codes are. The role description requires the codes to be handled "as per emergency code procedures". [¶ 8] Those procedures are Logan Hospital documents; they are not in the Respondent's List of Documents and will be called for. Pending their production, the Appellant will give evidence, as the operator who activated them, that at Logan Hospital in March 2024: a Code Blue was the call for a medical emergency in which a patient had suffered or was about to suffer cardiac or respiratory arrest, requiring the immediate attendance of the resuscitation team; a MET call was the call for the Medical Emergency Team to attend a deteriorating patient meeting the hospital's escalation criteria, with Neonatal and Paediatric MET calls for newborn and child patients; a Code Grey was the call for a security response to threatening or aggressive behaviour; and a Code Black was the call for a security response to a personal threat. Each code was received at the Switchboard, categorised by the operator, and paged to the response group for that category within the timeframe fixed by the procedure. The Australian Standard on which hospital emergency codes are based, AS 4083, uses the same colour scheme.

(h)(iii) The ordinary call load, and the Monday. The Appellant will give evidence, from his own monthly call statistics, that in the period of his graduated return in April 2025 he took between 269 and 444 calls on a single eight-hour shift, an average of about 360, and that the highest figure, 444, was on the one Monday afternoon shift in that sample (14 April 2025). He will give evidence that the Monday early shift is the peak of the week, because the weekend's backlog, the resumption of clinics and elective lists and the Monday admissions all arrive at the console together. The shift of 18 March 2024 was the Monday early shift, the first shift of the working week [¶ 227], and it followed a break of seven hours [¶ 284]. These matters are not admitted; they are the Appellant's own evidence and records, and will be proved by him.

(i) The leave of 19 March 2024. The Leave Takings Report produced by Metro South Health in respect of 19 March 2024 records the Leave Category as "Sick", the Leave Type as "Sick Leave", the Time Code as "SCK", the Leave Taken as 7.60 hours and the Leave Status as approved. [¶ 235]

The Respondent pleads that the Appellant took leave on 19 March 2024 but says this was paid leave, and that pursuant to clause 18.10 of the Award he is not entitled to fatigue leave because he was not performing overtime. [¶¶ 236 to 237] The Respondent's outlines (Part B.6) give notice that its payroll witness will say the Award's fatigue provision "applies to breaks around overtime, not between two ordinary shifts". On that account no recovery entitlement attached to a seven-hour break between two ordinary shifts, and the only protection was the roster itself.

(j) The enquiry of 8 April 2024, the delay, and the refusal of 1 May 2024. The review decision records: "You emailed Ms Taylor on 8 April 2024 and requested a review of your payment for the pay period between 8 March 2024 and 31 March 2024", noting the Award requirement as to a minimum ten-hour break. [¶¶ 242 to 243]

It records that she replied on 9 April 2024 and "added she also escalated your enquiry regarding fatigue leave to Human Resources to confirm policies around this", and that she "proceeded to acknowledge the importance of communication and requested that you raise your concerns with her as soon as they arise so she could action them sooner for you". [¶¶ 244 to 245]

It records that on 24 April 2024 the Appellant emailed expressing dissatisfaction that it had been more than two weeks without any response or overtime payment. [¶ 246]

The Respondent does not allege that any response was made to the fatigue enquiry between 9 April and 1 May 2024, an interval of 23 days. [¶¶ 249 to 250]

It records that she responded on 1 May 2024 stating that after a consultation with payroll and Human Resources the request for fatigue payment for 18 March 2024 would not be processed due to the existing 8-hour agreement, and noting that he was able to terminate that agreement going forward. [¶¶ 247 to 248]

It records that payroll was, as at 1 May 2024, still reviewing his entitlements; that the employer did not mention his shift on 18 March 2024; and that there was uncertainty between him and the employer regarding whether the 8-hour agreement continued to apply. [¶¶ 252 to 254]

(k) The employer's own fatigue policy. The review decision records that the policy regarding fatigue leave following weekends and rostered days off is dated June 2020. [¶ 239]

It records that "The policy notes that many problems can be overcome regarding fatigue leave by either not rostering the employee who is on call for the first shift of the following day and not rostering on call an officer who is rostered for the first shift on the following day." [¶ 240]

(l) The absence of fatigue risk management at the Switchboard. By letter of 5 June 2026, reference K-LM26/729, signed by the Chief Executive and addressed to the Commission, Metro South Health stated that the requested fatigue risk management documents "do not exist", that mandatory Fatigue Risk Management System training "only applies to health practitioners and clinical assistants" and that Switchboard staff are non-clinical. [¶¶ 263, 265] It stated that "The implementation of fatigue risk management assessment at Switchboard Logan Hospital occurred after 30 June 2024". [¶ 264] It stated that "there have been no 'consequential' changes to operating procedures over the period requested". [¶ 266] The Respondent does not allege that any fatigue risk assessment was conducted, that fatigue risk management training was provided, that fatigue risk management assessment was implemented at the Switchboard,

or that any change was made to the operating procedures of the Switchboard as a consequence of any employee complaint, at any time before 30 June 2024. [¶¶ 269 to 272]

(l)(i) The employer's own fatigue guideline. On 20 May 2024 the Director sent Human Resources the Queensland Health Fatigue Risk Management Systems Implementation Guideline, qh-gdl-401-3.3. [¶¶ 222 to 223] Ten days earlier she had written that she and the Manager had run through the Roster Risk Assessment Matrix and "we say at best there would be a rating of 11 which is moderate". [¶ 219] The Respondent disclosed the guideline, with that email chain, on 14 July 2025 (Tab 8A). The following passages are from that disclosed copy.

The guideline defines fatigue as "A state of impaired physical and/or mental performance and lowered alertness arising as a result or combination of physical and mental work, health and psychosocial factors or inadequate restorative sleep", and a fatigue risk management system as "An integrated set of management practices and procedures for monitoring and managing the risks posed to health, safety and wellbeing by fatigue". It defines "Defences in depth" as a "Hazard identification and risk control model that applies a series of layered mechanisms to minimise the occurrence of fatigue related incidents". [Guideline, glossary, p 29]

It provides that "Level 4 controls relate to error tolerance and containment" and that "Error tolerance relates to built-in redundancies that decrease the likelihood that a fatigued individual will make an error which may have catastrophic consequences." [p 21] It provides that "workers should be encouraged to report fatigue via the organisation's incident management system or other relevant hazard or incident reporting channels", and that reporting fatigue may constitute "a situation where a worker has identified that they are suffering from fatigue due to work-related factors". [p 22] It provides that determining whether fatigue contributed to an incident "is reliant on evidence such as rostering information, actual hours worked, fatigue leave, fatigue self-assessment scores and reporting of fatigue hazards". [p 23] It lists among performance indicators "fatigue risk management training compliance levels" and "payroll data (fatigue leave and overtime accessed)". [p 24, section 6.1] It lists among its related documents the Hospital and Health Services General Employees (Queensland Health) Award - State 2015 and the "I1 (QH-POL-171) Fatigue risk management policy". [p 28]

On consultation and training, the guideline states: "there is a legislative obligation to consult with workers in relation to hazard identification and the making of decisions about how associated risks can be managed. Consultation requirements for Queensland Health are described in Health, safety and wellbeing consultation standard (QH-IMP-401-2:2020)." It states that information, training and instruction "must have regard for: the nature of the work carried out; the nature of the risks associated with the work; the control measures implemented", and that "where a work unit or an occupational stream has been identified at risk of fatigue, the training provided must consider the above criteria". Its example training matrix (Figure 11) lists, as those to be trained, "Staff at risk of fatigue", "Rostering staff" and "Manager, supervisors, executives", with learning outcomes including "Rostering". [p 26, section 7.1] It states that successful implementation "relies strongly on effective communication", including "the training of relevant stakeholders so there is an understanding of the actions required of individuals to ensure the FRMS performs as intended". [p 25, section 7] It states that monitoring and assurance "can highlight situations where control measures may require modification" and provide "an opportunity to identify emerging issues and introduce new procedures and controls to mitigate risk". [p 25, section 6.2]

Its supporting documents include the "Fatigue risk assessment tool", the "Individual fatigue likelihood calculator", the "Samn-Perelli fatigue checklist", "Preventing and managing fatigue-related risk in the workplace (Workplace Health and Safety Queensland)", the Health, safety and wellbeing consultation standard (QH-IMP-401-2:2020), the Health, safety and wellbeing risk management standard (QH-IMP-401-3:2020) and the Health, safety and wellbeing policy (QH-POL-401:2020). [pp 27 to 28, section 9] Its legislation list includes the Work Health and Safety Act 2011, the Work Health and Safety Regulation 2011, and the "Managing the risk of work-related psychosocial hazards Code of Practice". [p 27, section 8] It defines a "Worker" by reference to section 7 of the WHS Act, including "An employee", and states that "The Department of Health and each of the HHSs are considered to be PCBU's". [pp 29 to 30, section 10] Its version control records that version 1.0 of 10 November 2021 replaced the "Fatigue Risk Management System Resource Pack December 2018 v. 3". [p 30, section 11] Its references include "Queensland Ombudsman (2006). The Neville Report" and Ziebertz and others (2015) on "The Relationship of on-call work with fatigue, work-home interference, and perceived performance difficulties". [pp 30 to 31, section 12] Each page of the disclosed copy carries the footer "Managing the risk of psychosocial hazards at work Code of Practice 2022 applies 1 April 2023".

The guideline therefore treated the Award under which the Appellant was employed as a supporting document, treated every employee of a Hospital and Health Service as a worker to whom it applied, called for consultation with workers on fatigue hazards, called for training of rostering staff and managers, and had been preceded by a resource pack since December 2018. The Chief Executive's letter states that fatigue risk management assessment at the Switchboard was implemented after 30 June 2024 and that the mandatory training "only applies to health practitioners and clinical assistants". [¶¶ 264 to 265] The Respondent does not allege that any fatigue risk management training was provided to the Appellant, or that any fatigue risk assessment of his position was conducted, before 30 June 2024. [¶¶ 269 to 270]

The guideline the Director sent to Human Resources on 20 May 2024 is the guideline the Chief Executive's letter of 5 June 2026 says was implemented at the Switchboard only "after 30 June 2024" [¶ 264], and the training for which "only applies to health practitioners and clinical assistants" [¶ 265]. The fatigue risk of the Appellant's roster had been assessed by the Director and the Manager as moderate on 10 May 2024 [¶ 219]; no fatigue risk assessment of his position is alleged to have been conducted before 30 June 2024 [¶ 269].

(m) The findings in the decision under appeal. The review decision states: "The break between the shift on 17 March 2024 and 18 March 2024 equated to 7 hours." [¶ 258]

It states that even if the Appellant was allowed to leave early on 17 March 2024 as suggested by the employer, he left a maximum of thirty minutes early, which meant he still did not receive a minimum 8-hour break. [¶ 259]

It states: "Based on this, I find the rostering of these two shifts amounted to unreasonable management action given that it was in direct contradiction to the award and the 8-hour agreement." [¶ 260]

It states, under the heading "Conclusion", that the Appellant "sustained a personal injury of a psychological nature". [¶ 261]

It states that his injury "arose out of employment, to the extent that it arose out of factors 2, 3 and 4, where employment was a significant contributing factor". [¶ 262]

Medical linkage - Stressor 3. Tab M1 records the 1 July 2024 report that the roster was being manipulated so the Appellant worked "lates then earlies". Tab M4 later records workplace stress involving management and rostering. Tab M8 is the review decision recording the seven-hour break and the finding that the rostering of the 17 and 18 March shifts was unreasonable management action. Tab M7 is relied upon only for continuing effect and capacity in 2026. The medical material is not relied upon to prove the roster events; those events are proved by the admitted roster, policy, correspondence and review-decision facts above.

Cumulative medical case. The medical material does not assign a separate diagnosis to each pleaded event. The Appellant relies on one psychological injury said to arise from the composite course. Repetition of an admitted fact under more than one stressor is therefore functional, not duplicative: it allows each stressor to be determined on its own while preserving the cumulative course described to the treating practitioners.

3. Subsequent conduct, and the contemporaneous record

3.1 Post-injury course - context only, not causation. The work capacity certificates certify no functional capacity from 1 July to 6 October 2024. The Appellant proposes to give oral evidence that, during certified leave, Ms Reese contacted him and meetings were scheduled. His outline of evidence also gives notice that his employment was later ended under abandonment provisions with an effective separation date of 20 September 2024, and that he was later reinstated. The effective separation date therefore falls within the certified no-capacity period. The later October communication or processing of the abandonment outcome is not pleaded as having occurred while the final certificate was still current. These matters are not part of the admitted-fact core and are not relied upon as causes of the injury said to have arisen by June 2024. They are retained only as post-injury context, continuing-effect/capacity history and matters notified for oral evidence. Any later WorkCover, income-protection, return-to-work or employment dispute is treated in the same way unless separately admitted and relied upon for another defined purpose.

3.2 What the Appellant provided to WorkCover Queensland, and when. The Respondent's amended List of Documents of 14 August 2026 records that the Appellant provided to WorkCover Queensland, at item 12, an email of 12 July 2024 with an attachment described as "Event overview - undated"; at item 14, an email of 18 July 2024 with an attachment described as "Witness statement Carolyn Jeffrey"; at item 25, an email of 29 August 2024 described as regarding the after hours on call change, with an attachment described as "Email: After hours on call process"; at item 26, an email of 30 August 2024 described as regarding failure to consult, with an attachment described as "Email: Task change switchboard - 19/04/2024"; and at item 27, an email of 30 August 2024 described as regarding failure to consult, with an attachment described as "Email: MASPER process - 09/05/2024". It further records, at item 16, an email from Ms Carolyn Jeffrey to WorkCover Queensland of 1 August 2024 described as a follow up statement. The documents at items 25, 26 and 27 are the three directives pleaded at Stressor 1(e) and Stressor 1(g) above. They were provided by the Appellant to WorkCover Queensland in August 2024, before any decision on his claim and more than two years before this pleading.

3.3 The particulars bundle of 11 August 2026. The Respondent's amended statement of facts and contentions states at paragraph 11 that it does not admit the allegations in Stressor 1(a) of the Appellant's statement "because there are no particulars or details to respond to". On 11 August 2026 the Appellant served a bundle titled "Stressor 1(a) - Particulars support bundle", comprising 30 pages and six tabs, each stating a particular of that stressor and enclosing the documents recording it.

Particulars: paragraphs 274 to 282 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

4. The intervals The following intervals are computed from dates and times admitted by the Respondent. They are matters of arithmetic and do not depend on the truth of any statement recorded in a document.

4.1 Between the MASPER Registrar's first report of 3 May 2024 at 3:06 pm and the first reply of 9 May 2024 at 9:20 am: five days, eighteen hours and fourteen minutes. Her second report was sent on the fifth calendar day after the first; the reply was sent on the sixth.

4.2 Between the first email of the Integrated Respiratory Service of 15 May 2024 at 11:47 am and its second of 20 May 2024 at 11:03 am: the fifth calendar day. Between that first email and the Appellant's escalation of 20 May 2024 at 2:05 pm: five days, two hours and eighteen minutes. Between the Service's second email and that escalation: three hours and two minutes. Between the escalation and the reply at 4:30 pm: two hours and twenty-five minutes, that reply being sent two hours after the conclusion of the office hours stated to all Switchboard staff on 17 May 2024.

4.3 Between the Appellant's fatigue enquiry of 8 April 2024 and the response of 1 May 2024: 23 days, during which the Respondent does not allege that any response was made.

4.4 Between the third submission of the special pandemic leave request at 11:07:28 on 29 February 2024 and its approval at 11:21:03 the same morning: thirteen minutes and thirty-five seconds. Between the creation of the draft on 20 February 2024 and the final approval on 1 March 2024: ten days, four hours, forty minutes and twenty seconds.

4.5 Between the removal of database access on 18 July 2023 and 18 June 2024: eleven months, during which the Respondent does not allege that access was restored. Between 23 August 2023 and 17 May 2024: nine months, during which the Respondent does not allege that fixed office hours were stated to the Switchboard staff.

Particulars: paragraphs 62 to 64, 92, 97 to 98, 102 to 103, 124 to 125, 249 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

5. Matters the Respondent does not allege, and documents it does not list The following are admitted as to the state of the Respondent's amended statement of facts and contentions of 13 May 2026 and its amended List of Documents of 14 August 2026. They are not admissions as to the truth of the contents of any document, and the qualification in the Respondent's letter of 8 September 2026 does not reach them.

5.1 As to the directory and the changes to process. That access to the database was restored to the Appellant before 18 June 2024; that any document records consultation with Switchboard operators before the change communicated on 15 April 2024; that agreement under clause 6.2 of the Award was obtained, or a ballot conducted, before that change.

5.2 As to what followed the reports at the console. That any communication was sent to Logan Switch or the Switchboard staff about the occasions reported on 3 and 8 May 2024 before 10:15 am on 9 May 2024; that any response was made to the Integrated Respiratory Service before 20 May 2024, or before 2:05 pm on that day; that any document created on 20 May 2024 records the discussion referred to in the reply of that afternoon; that any communication was sent by either named person to that Service on that day; that the contact-details document was amended on or before 20 May 2024; that the Switchboard staff were notified of the modifications made to it on 22 February 2024; or that the Appellant was rostered to work after 2:05 pm on 20 May 2024.

5.3 As to the office hours. That fixed office hours were stated to the Switchboard staff at any time between 23 August 2023 and 17 May 2024.

5.4 As to the Communication Book. That the entry removed, or its author, is identified; that the removed page has been located or that any copy of it exists; that any entry made by the Appellant remains in the book; or that the book ceased to be in the possession of Metro South Health during the Appellant's employment. No proposition about whether the book is listed in the amended List of Documents is relied upon in this paragraph; fact 154 was not admitted.

5.5 As to the special pandemic leave. That either person recorded in the leave history was a Band 9 delegate at any relevant time; the identity of the person who exercised the sub-delegated power in respect of Process Reference 15480560; or that any other Switchboard employee was required to submit such a request personally through myHR in February 2024.

5.6 As to the roster and fatigue. That the Switchboard Manager got in touch with the Appellant about alternative shifts after the meeting of 16 April 2024; that the fatigue toolkit was reviewed or feedback given before 18 June 2024; that the consecutive shifts of 17 and 18 March 2024 arose from a staff-initiated shift swap; that any response was made to the fatigue enquiry between 9 April and 1 May 2024; that any fatigue risk assessment was conducted, that any fatigue risk management training was provided, or that fatigue risk management assessment was implemented at the Switchboard, before 30 June 2024; or that any change was made to the operating procedures of the Switchboard as a consequence of any employee complaint over the period 1 December 2023 to 30 June 2024.

5.7 As to the Appellant's performance and conduct. That he was subject to any disciplinary process, or that his work performance was the subject of any formal performance management process, before 18 June 2024; nor does the Respondent describe any communication with him before that date as a warning. Paragraph 27 of its statement of facts and contentions does not identify, by particular, date, document or cross-reference, the management action relied upon for the contention in that paragraph.

Particulars: paragraphs 16, 21 to 22, 25, 55, 69, 73, 90, 93, 99, 105 to 110, 129 to 131, 142, 150 to 153, 166, 180 to 181, 213, 216, 234, 250, 269 to 273, 300 to 303 of the notice to admit facts served 28 August 2026, admitted 8 September 2026. Paragraph 154 of that notice is not admitted.

6. The Respondent's notified evidence, served 24 September 2026

On 24 September 2026 the Respondent served outlines of the evidence of Ms Taylor, Ms Reese, Ms Wright and Ms Earl. An outline is not evidence. The statements below are pleaded as notice of the Respondent's case, so that the issues are defined before hearing, and are relied upon only as the Respondent's own description of the matters to which they relate. The Appellant will ask each witness to confirm them.

6.1 Ms Taylor. The outline states that she "was the Switchboard Manager, Logan Hospital, and the Appellant's direct line manager, in an acting capacity, from around April 2023 to 1 July 2024 supervising rostering, attendance and shifts"; that she "Confirms the seven-hour break between shifts on 17-18 March 2024 was the result of human error"; that she "Confirms the AVAC correspondence of 3, 21 and 28 May 2024, in which she was involved in emails about correcting the Appellant's shift payments"; that she "sought advice from HR, who confirmed the agreement was on file and continued to apply", no document recording that advice being identified; and that, acting on HR's advice, she "removed the email from the shared switchboard inbox herself", being the email of 15 May 2024 at Stressor 1(i).

6.2 Ms Reese. The outline states that the email of 15 May 2024 was "asking that Ms Taylor's office hours be shared with the whole department"; that she emailed the Appellant "at 4:12pm" and "again in materially the same terms at 6:23pm" asking him to retract it, on the basis that it "could have been raised with Ms Taylor or Ms Reese directly rather than sent to the whole team"; that on 7 August 2023 the Appellant raised "concerns about rostering fairness" and "a claimed breach of the enterprise agreement's roster-display requirements", and that she "advised him of his option to lodge a formal grievance with HR"; and that her understanding that the eight-hour agreement continued to apply after the move to full-time employment was "through Ms Taylor".

6.3 Ms Wright (Payroll). The outline states that the pay issues raised "were investigated and, where a discrepancy or error was found, corrected"; that a public holiday allowance "must be approved and submitted by the line manager on an AVAC"; that the agreement of 17 June 2020 reduced "his break between shifts from ten hours to eight"; that payroll "does not itself assess an employee's entitlement under them, or under any change in employment status"; and that, when the system did not apply an additional payment for the break of 17 to 18 March 2024, the advice returned was "that the Award's fatigue provision applies to breaks around overtime, not between two ordinary shifts, and that what had occurred was better characterised as a rostering practice issue for the line manager rather than an unpaid entitlement".

6.4 Ms Earl (Payroll). The outline states that the Appellant "uploaded his Special Pandemic Leave application together with a supporting attachment on 20 February 2024 at 11:41am"; that it was declined on 21 February "on the stated basis that a minimum five-day period of leave was required" and on 29 February "for want of an attached statutory declaration or evidence of a positive test"; that "On payroll's own review of the system's attachment history, however, the supporting document had in fact been attached from the original 20 February submission onward, meaning the second decline also appears to have been made in error"; that it was then "approved that day by the delegate and manager" and that the Appellant "continued to be paid, throughout the period this was being sorted out"; that for the fortnight including 30 March 2024 an entry was first processed so as to "trigger an automatic hours reduction on his payslip" and was "later corrected"; and that from the roster the break of 17 to 18 March 2024 "was only seven hours, that he was rostered on ordinary hours rather than overtime at the time, and that he took paid sick leave on 19 March 2024".

6.5 Matters the outlines do not address. No outline addresses the removal of database access or its restoration; the Contact & Number Changes book; any consultation preceding the change notified on 15 April 2024; the reports of the MASPER Registrar of 3 and 8 May 2024; the emails of the Integrated Respiratory Service of 15 and 20 May 2024; the amendment of the clinic contact-details document; the fatigue enquiry of 8 April 2024 or the interval to 1 May 2024; the email to Human Resources of 10 May 2024; or the absence of fatigue risk management at the Switchboard before 30 June 2024. No medical or expert evidence is notified.

PART C - CONTENTIONS

1. The onus, accepted. The Appellant accepts that he bears the onus of establishing on the balance of probabilities that he sustained an injury within s 32 and that the exclusion in s 32(5)(a) does not apply to the injury proved. The reasonableness inquiry is objective. *Prizeman v Q-COMP* [2005] QIC 53; (2005) 180 QGIG 481 states that the reality of the employer's conduct, rather than the worker's perception of it, is the relevant focus. The contrary contention in the

Amended Form 9A filed 7 April 2026 is withdrawn.

2. The admitted record supplies the factual basis for that assessment. The Appellant does not ask the Commission to infer unreasonableness from characterisation alone. The Respondent has admitted, to the extent recorded in its 8 September 2026 response, the documentary propositions set out in Part B, including that database access was removed and is not alleged to have been restored before 18 June 2024; that no document records consultation with Switchboard operators before the 15 April 2024 change; that the nine reported occasions of calls reaching the wrong medical team preceded the first identified communication to Switchboard staff about them by nearly six days; that the 20 May record contains the absences identified at Stressor 1(h); and that the employer's 5 June 2026 letter states the identified fatigue records do not exist, the mandatory training applied only to health practitioners and clinical assistants, fatigue assessment at Switchboard began after 30 June 2024, and there were no consequential changes to operating procedures over the requested period.

3. Compensable injury - s 32(1). The Appellant contends that he sustained a personal injury of a psychological nature arising out of, or in the course of, his employment, with employment a significant contributing factor. The injury is the diagnosed condition identified in Part A. It is pleaded as arising from the composite course in Part B.2, not from a single-day event. The Respondent's review decision recorded both a psychological injury and employment as a significant contributing factor, while rejecting the claim under s 32(5).

4. Section 32(5)(a) - operational conditions said not to be management action. The Appellant contends that some circumstances in Stressor 1 are conditions in which the work was required to be performed rather than management action: the position remained responsible for accurate contact and paging information while operators could not directly amend the database; calls, including a MET-call enquiry, were misdirected to the wrong medical team; a clinical service reported that it could not assist patients or staff because of a wrong directory destination; and operational complaints were, on the employer's own account, managed in the manner recorded in its 5 June 2026 letter. The Commission is asked first to classify the circumstances before applying the exclusion.

5. Section 32(5)(a) - if management action, whether reasonable and taken in a reasonable way. To the extent the pleaded circumstances are management action, the Appellant relies on the admitted sequence, timing and documentary omissions in Part B. In particular: the 15 April process change was effective on the day it was notified and no document of prior consultation is listed; remuneration corrections across identified fortnights were directed back through the line-manager process and one later claim remains recorded as "Part Completed" while two identified February AVAC references do not appear in the report; the paid Special Pandemic Leave entitlement in Stressor 2(f) required three submissions, was declined twice, and the Respondent later pleaded that the supporting attachments were in fact present when the later decline occurred; the fatigue enquiry was escalated to Human Resources and no response is alleged for 23 days; and the roster decision described in the 10 May email was not to be disclosed to the Appellant until the implementation plan was issued. The Appellant relies on the intervals and negative pleading/document-list admissions in Parts B.4 and B.5.

5A. The Respondent's notified evidence is consistent with the admitted record. On the Respondent's own outlines (Part B.6): the seven-hour break was "the result of human error" and, on the advice returned within payroll, "a rostering practice issue for the line manager"; the agreement of 17 June 2020 reduced the break "from ten hours to eight", so that a seven-hour break fell below both; no recovery entitlement attached to a short break between two ordinary shifts; the second decline of the paid Special Pandemic Leave request "appears to have been made in error"; and pay corrections were the line manager's to submit. The Appellant contends that the rostering of 17 to 18 March 2024 was, on the Respondent's own description, an act of the line manager in rostering; that it is management action; and that it was not reasonable management action taken in a reasonable way, for the reasons in Stressor 3. The matters the outlines do not address (Part B.6.5) stand on the admitted facts.

6. Global assessment where appropriate. *Delaney v Q-COMP Review Unit* [2005] QIC 11; (2005) 178 QGIG 197 supports a global evaluation where multiple aspects of management action are sufficiently connected by subject matter, time and the persons involved. The Appellant relies on that approach only where the Commission considers the connection sufficient. Each pleaded stressor is also stated independently so that the reasonableness question can be answered without importing a fact from another stressor.

7. The ultimate statutory question concerns the injury as a whole. *Workers' Compensation Regulator v Langerak* [2020] ICQ 2 at [86] states that the ultimate question under s 32(5)(a) is whether the injury, not any one stressor, arose out of or in the course of reasonable management action taken in a reasonable way, requiring the reasonable-management-action factors to be weighed against other employment factors in light of the whole evidence. *Simon Blackwood (Workers' Compensation Regulator) v Mahaffey* [2016] ICQ 10 is consistent with the proposition that the presence of reasonable management action among the causal circumstances does not by itself resolve the statutory question where other employment factors also caused the disorder. The review decision's finding that the 17 and 18 March 2024 rostering was unreasonable management action is relied upon as an admitted document, not as a binding determination.

8. The effect of the admissions. The admissions are made for this proceeding under rule 49 and are relied upon as evidence of the existence and terms of the documents admitted, and of the matters the Respondent's statement of facts and contentions and List of Documents do not contain. The Appellant does not contend that any admission determines the reasonableness of any conduct; that is a matter for the Commission.

9. The hearing de novo. The Respondent says no finding in Review Decision 69983 binds the Commission. That is accepted, and it applies to the whole of the decision. The conclusion under appeal is a conclusion of that decision, reached upon the findings recorded in it.

10. The Respondent's pleaded case. Paragraph 27 of the amended statement of facts and contentions does not identify, by particular, date, document or cross-reference, the management action relied upon. No disciplinary process, formal performance management or communication described as a warning before 18 June 2024 is alleged.

Particulars: paragraphs 238, 295, 300 to 303 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

PART D - THE QUESTIONS FOR DETERMINATION

- 1.** Did the Appellant sustain a personal injury of a psychological nature?
- 2.** Did that injury arise out of, or in the course of, his employment, and was his employment a significant contributing factor to it?
- 3.** Which of the circumstances pleaded in Part B.2 constitute management action taken by or on behalf of the employer in connection with the Appellant's employment, and which are conditions in which the work was required to be performed?
- 4.** In respect of those circumstances that are management action, was that action reasonable and was it taken in a reasonable way?
- 5.** Does section 32(5)(a) of the Act operate to exclude the Appellant's psychological disorder from the definition of injury, having regard to the circumstances found not to be management action and to any management action found not to have been reasonable or not to have been taken in a reasonable way?

PART E - ORDERS SOUGHT

- 1.** That the appeal be allowed.
- 2.** That the decision of the Respondent dated 24 October 2024, Review Decision 69983, be set aside.
- 3.** That it be declared that the Appellant sustained an injury within the meaning of section 32 of the Workers' Compensation and Rehabilitation Act 2003, being a psychological injury arising out of, or in the course of, his employment with Metro South Hospital and Health Service, employment being a significant contributing factor to that injury.
- 4.** That the Appellant's application for compensation be accepted, and that the matter be remitted to the Respondent for assessment of the Appellant's entitlement to compensation consequent upon that acceptance.
- 5.** Such further or other order as the Commission considers appropriate.
- 6.** That the Respondent pay the Appellant's costs of the appeal under section 558(3) of the Act and section 191 of the Workers' Compensation and Rehabilitation Regulation 2025, including an award under section 191(3) of up to 1.5 times the amounts under section 191(2)(a), to be assessed failing agreement; alternatively, that costs be reserved.

PART F - COSTS

- 1.** Costs of the hearing are in the Commission's discretion: section 558(3) of the Act; section 191(1) of the Workers' Compensation and Rehabilitation Regulation 2025. Costs ordinarily follow the event.
- 2.** If the appeal is allowed, the Appellant will seek an order that the Respondent pay his costs of the appeal, assessed under section 191(2): (a) counsel's or solicitor's fees, for any period in which the Appellant is legally represented, at the costs stated in the Uniform Civil Procedure Rules 1999, schedule 2, part 2, scale C; (b) costs in relation to witnesses at the allowances stated in the Uniform Civil Procedure (Fees) Regulation 2019, part 3, including the fees and expenses of the medical practitioners called under paragraph B.1.11; and (c) any costs in relation to bailiffs.
- 3.** The Appellant will contend that the amounts under section 191(2)(a) are inadequate and will seek an award of up to 1.5 times those amounts under section 191(3), in total or in relation to any item, having regard to: (a) the work involved, being a record of 303 facts of which 298 are admitted, the documents at Annexure A to the notice to admit, non-party disclosure obtained under rule 64G, two notices to admit facts and a notice to admit documents, and the outlines of four witnesses served by the Respondent; and (b) the importance, difficulty and complexity of the matter, being an appeal concerning a psychological injury under section 32(5)(a) which requires the classification of each circumstance pleaded, a finding on the reasonableness of each that is management action, and the weighing of the whole.

4. On the exercise of the discretion under section 558(3), the Appellant will also rely on the course of the proceeding, including that paragraph 27 of the Respondent's amended statement of facts and contentions identifies no management action [¶ 303], that the Respondent has notified no medical or expert evidence (Part B.6.5), and that the Respondent's own review decision found the rostering of 17 and 18 March 2024 to be unreasonable management action [¶ 260].

5. To the extent that the fees and expenses payable to a medical practitioner, or to any witness who gives evidence of a professional nature, exceed the costs awarded in relation to that witness, the Appellant will apply to the Respondent under section 192 of the Regulation for payment of an additional amount.

6. The Appellant reserves the right to rely, on the question of costs, on any offer made without prejudice save as to costs.

Dated: 2026

Cory Lea ShepherdAppellant, self-represented